

CHANGES IN SKIN COLOUR

Is the skin yellow, too dark, too light or absent of colour?

Yellow skin

Jaundice likely

Refer urgently the patient with jaundice and any of:

- Temperature $\geq 38^{\circ}\text{C}$
- Hb < 12 (woman) or < 13 (man)
- BP $< 90/60$
- Severe abdominal pain
- Drowsy or confused
- Easy bruising or bleeding
- Pregnant
- Alcohol dependent $\rightarrow 142$ or recent alcohol binge (≥ 4 drinks¹/session)
- Using any medication² or illegal drugs

- Send blood for ALT, ALP, total bilirubin, full blood count, INR, hepatitis A IgM, HBsAg.
- Advise to return if worsens.
- Review with results within 2 days:

Refer if ALT ≥ 200 , INR ≥ 1.5 , ALP raised out of proportion to ALT, Hb < 12 (woman), Hb < 13 (man) or plts < 150 .

Hepatitis A IgM positive

Patient has acute hepatitis A infection

- Notify.
- Educate that infection will resolve by itself and no specific treatment needed. Advise strict handwashing practises, especially before handling food and after using toilet. Avoid alcohol and paracetamol whilst ill.
- Check HBsAg results $\rightarrow 120$.
- If nausea/vomiting and unable to tolerate fluids, refer.

Hepatitis A IgM negative

Check HBsAg results $\rightarrow 120$.

Dark patches

Where are patches on body?

Lower legs

Red-brown discolouration. May have breaks in skin or ulcers, spidery veins.



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Venous stasis likely

- Encourage exercise.
- Advise elevating leg when possible and to avoid prolonged standing.
- Apply compression bandage from foot to knee.
- Assess CVD risk $\rightarrow 127$.
- Give foot care advice $\rightarrow 66$.
- If ulcer $\rightarrow 75$.

Face

Flat, brown patches on cheeks, forehead and upper lip



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Melasma likely

- Hormones and sunlight will worsen melasma:
- Advise to apply sunscreen daily and avoid sun exposure to face.
- Avoid oral contraceptive, rather use a different method $\rightarrow 154$.
- Advise patient:
 - If pregnant, may take up to 1 year after pregnancy to resolve.
 - Often difficult to treat and may never completely resolve.
- If not responding to above and intolerable, refer.

Light patches

Light or dark patches with fine scale. Usually on trunk, neck and upper arms.



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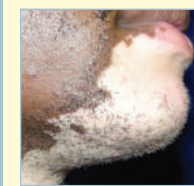
Tinea versicolor likely

- Advise to wear cool clothing in hot weather to reduce perspiration.
- Apply **selenium sulphide 2.5%** suspension. Lather on affected areas:
 - Apply daily for 3 days: leave on for 30 minutes then wash off *or*
 - Apply weekly for 3 weeks: leave on overnight then wash off.
- May take months for colour to return. Absence of scale indicates adequate treatment.
- Recurrence is common. Re-treatment may be needed.

Absence of colour

Is absence of colour patchy or generalised?

Patchy



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Vitiligo likely

- Refer to dermatologist.
- Advise to avoid excessive sun-exposure and apply **titanium dioxide** ointment/cream (UV block) at least 15 minutes before going into sun between 10am and 3pm. Some sun-exposure is beneficial before 10am and after 3pm.

Generalised

Present from birth. Involves skin, hair and eyes.

Albinism likely

- Advise to avoid sunburn:
 - Avoid sun exposure, especially between 10am and 3pm.
 - Apply **zinc oxide** ointment or **titanium dioxide** ointment/cream (UV block) daily at least 15 minutes before going into sun. Reapply 2 hourly if in the sun.
 - Use sun hat and sunglasses and wear long-sleeves.
- Refer to dermatologist and ophthalmologist.
- If any skin lesions develop, especially in sun-exposed areas, refer to exclude skin cancer.

If diagnosis uncertain, discuss/refer.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ²If on atazanavir $\rightarrow 116$.